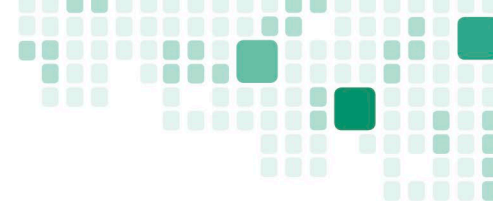




EMHS Mental Health Bed Access, Capacity, and Escalation Framework

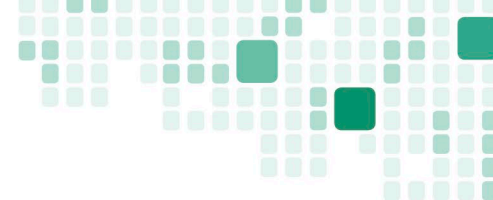




Contents

Document Control	1
Purpose	2
Definitions	2
Principles	3
Roles and responsibilities	4
EMHS Patient Flow Coordinator	4
Bed Flow Manager	5
Nurse Unit Managers	5
Psychiatric Liaison Nurse (PLN) / Mental Health Liaison Team (MHLT)	6
Community Mental Health Teams	6
Consultant	6
Operations Hub Manager	6
Head of Service - Psychiatry / Medical Co-Director	7
Executive Director Mental Health	7
Statewide Nurse Director Mental Health Patient Flow	7
Statewide Mental Health Medical Director	8
Overview of Beds	8
Access pathways and process to services	8
Required Documentation	8
Bed Flow Process	9
Demand & Capacity	10
BRAG System	10
Capacity Making Strategies	12
Escalation	14
System wide Response	14
Catchment Area Considerations	14
Escalation Pathway	16
Bed Usage	17
Use of Leave Beds	17
Use of an Absent With Out Leave (AWOL) bed	17
Over census	18
Clinical Prioritisation	18
Appendix 1: WACHS catchment areas	20





Document Control

Document history:

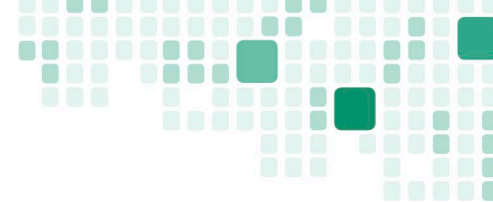
Version	Date	Author	Reason for update
0.7	25/01/19	EMHS Mental Health Patient Flow Project	Initial working draft version approved by EMHS Executive Mental Health 20/12/18
1	02/07/19	EMHS Mental Health Patient Flow Project	Following approval of Mental Health Bed Access, Capacity and Escalation Statewide Policy 2019
2	10/10/19	EMHS Mental Health Patient Flow Project	Preparation for opening of MHEC, RPH
3	17/07/25	EMHS Mental Health Strategy	Alignment with Statewide Mental Health Bed Access, Capacity and Escalation Policy
3.1	25/08/25	EMHS Mental Health Strategy	Alignment with Statewide Mental Health Bed Access, Capacity and Escalation Policy
3.2	29/10/25	EMHS Mental Health Strategy	Minor wording updates to incorporate feedback from Aboriginal Health staff. Minor wording updates to incorporate feedback from Site Executive.

Distribution review checklist

Consultation:	EMHS Mental Health Patient Flow Implementation Group	1/7/19
	EMHS Patient Flow Framework Working Group	30/08/25
Authorised by:	EMHS Executive Director Mental Health	29/10/25

Information contained in this document is correct at the time of publication but may change due to further planning considerations as they become evident.





Purpose

This document outlines processes to manage bed flow across EMHS Emergency Departments (ED), and mental health community services and inpatient settings, including St John of God Midland Public Hospital (SJGMPH).

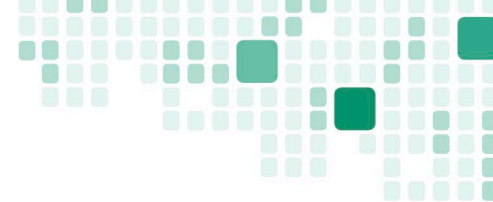
This document must be read in conjunction with the Department of Health [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#), which can be found on the [State Health Operations Centre \(SHOC\) hub page](#).

Note that this document references use of the Enterprise Bed Management (EBM) system. EBM is used within the Armadale Kalamunda Group (AKG) and Royal Perth Bentley Group (RPG), while St John of God Midland Public Hospital (SJGMPH) utilises webPAS in place of EBM. Where EBM is referenced, the equivalent webPAS process should be followed for SJGMPH.

Definitions

Term	Definition
Absent without Leave (AWOL)	For the purpose of this policy, the definition of AWOL is related to the inpatient setting where a bed would be available in the AWOL person's absence. Under section 97 of the <i>Mental Health Act 2014</i> (WA) (MHA), a person is considered AWOL from an authorised hospital if they leave without permission, when they are: • under an inpatient treatment order (i.e., the person is an involuntary inpatient and under Form 6A/B/C) and: a. leave has not been granted; or b. they fail to return to the hospital after their leave has been cancelled or expired
AWOL Bed	An Absent Without Leave Bed (AWOL) is an available bed when the person has been declared AWOL for > 12 hours.
Leave Bed	A leave bed is an available bed when an admitted person is on prolonged leave (overnight or longer). The WebPAS transaction is to occur to put the person 'out on leave'.
Outliers	In circumstances where demand outweighs bed availability it may be necessary to outlie a person to an inpatient bed outside of their treating specialty. For example, outliers may include adults who are outlied in an older adult bed, or vice versa.





Over census	An over census bed is an additional inpatient over the number of open and staffed beds in an inpatient unit. The over census person must be recorded against the 'OC' field in WebPAS.
Secure bed	EMHS defines a bed as secure when it meets all of the following conditions: • located in an authorised mental health ward • located in a ward with an entry/exit air lock • staff controlled access to the ward; doors are locked and can only be opened by authorised staff • located in a ward which meets required anti-ligature standards • located in a ward with environmental design that supports safe and appropriate care for high acuity populations • supported by nursing ratios which support higher acuity
Systemwide response	Coordinated response from all HSPs to address periods of peak demand for mental health beds and all HSP options have been confirmed as exhausted. Sites will be notified by the Statewide Nurse Director Mental Health when Systemwide response is activated. Refer to Statewide Mental Health Bed Access, Capacity and Escalation Policy for further details on Systemwide response including scenarios for activation.

To support a person-centred care approach, EMHS endeavours to use inclusive, person-first language such as 'person receiving care' or 'people accessing mental health services'. Within this document, the term 'patient' may be used when referring to role titles, when referencing other documents, or for clarity as required.

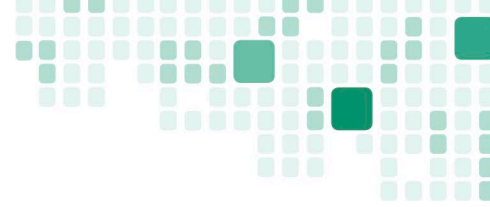
Principles

Mental Health bed flow within the EMHS is based on the following principles which will be upheld at every aspect of the care journey.

Care Experience

- All processes for bed flow must hold the person receiving care at the centre of care delivery
- The focus should be on keeping the person close to their support people wherever possible.
- For Aboriginal people, additional family and cultural supports may need to be engaged (e.g., family, nominated community members, Aboriginal Health Liaison Officers, and external Aboriginal health services).
- People attending the Emergency Department (ED) for mental health care will be managed as any other patient in the ED and are included in the WA Emergency Access Target (WEAT) requirement.





- Where possible, people should be directly transferred from ED to the clinical area most suitable to their needs. On occasion, a person may need to be transferred elsewhere at the discretion of the clinical team.

Bed Flow within EMHS

- EMHS will be responsible for meeting demand for beds with clear escalation processes
- Communication of a person's requirement for admission is an essential point in ensuring care is appropriate and not delayed
- Communication between services in EMHS will be collaborative and respectful
- EMHS mental health sites (excluding SJOGMPH) will utilise EBM to provide accurate visibility of capacity and improved communication between areas.
- EMHS patient flow will support oversight and coordination of referrals.
- Mental Health Business Rules for EBM apply to all metropolitan public mental health services (excluding SJOGMPH). EBM Business Rules can be accessed [here](#).

Roles and responsibilities

EMHS Patient Flow Coordinator

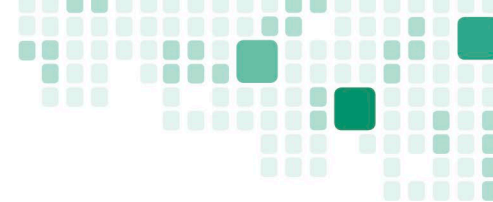
Mental Health Patient Flow Coordinators will be embedded within HSPs, and meet the requirements outlined in the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#). The EMHS Mental Health Patient Flow Coordinator reports to the Operations Hub Manager, with escalation to Mental Health Co-Directors.

The EMHS Mental Health Patient Flow Coordinators are available 0700 to 1700, seven days a week and fulfils the following responsibilities:

- Works with Bed Flow Managers on site to view and review collective HSP capacity, while ensuring line of sight beyond individual hospitals
- Provide updates to relevant medical lead/s
- Functions as a representative of the systemwide response
- Assists in dispute resolution process within EMHS by collating and providing additional information to decision makers
- Facilitates intra and inter HSP movement including transfers and repatriation where barriers present
- Updates the Statewide Nursing Director Patient Flow and other HSP Mental Health Patient Flow Coordinators throughout the day on EMHS bed status
- Maintains awareness of all HSPs (including WACHS) mental health bed demand and capacity via the [Assertive Mental Health Patient Flow Beds Live Dashboard](#) (the dashboard), or access to PSOLIS and iSoft, and through Patient Flow contacts if the dashboard is unavailable

As a central function the Mental Health Patient Flow Coordinator/s act to facilitate and coordinate bed flow across metropolitan HSPs and WA Country Service during times of high demand.





Bed Flow Manager

Each EMHS site will have a position that fulfils the role of a site Bed Flow Manager. At each of the EMHS sites, this role is carried out by the below positions:

- Bentley Health Service - Referrals Coordinator or After Hours CNS
- Armadale Health Service Gargangara - Nurse Unit Manager (NUM) / Associate NUMs or After Hours CNS
- St John of God Midland Public Hospital - Bed Flow or Mental Health Liaison Nurse (MHLN)
- Royal Perth Hospital Walbirring Karlup - Respective NUM for inpatient service (2K, MHEC, Mental Health Unit) or Afterhours Emergency Department Mental Health Liaison Team (ED MHLT)

The Bed Flow Manager must follow the Mandatory Business Rules for Bed Management as per Appendix 3 of the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#).

Within EMHS, Bed Flow Managers, in partnership with the multidisciplinary team, are responsible for:

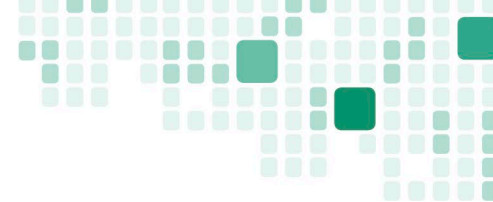
- Communications to referrers and between sites
- Ensuring patient flow information is recorded in EBM (AKG and RPBG) and webPAS (SJOGMPH)
- Prioritising admissions in accordance with acuity and risk
- Regular communications with EMHS Patient Flow Coordinator especially regarding areas such as transport, HSP negotiations, and care needs in ED (e.g., Nurse specialising or Aboriginal Health Liaison Officers)
- Regular communications with ward NUMs and responsible consultant
- Supporting sites to review demand and prioritise admissions and discharges, preventing escalation of BRAG status
- Ensuring all admissions and discharges are planned and consideration for the presenting risk, clinical needs for admission or for discharge are documented, and a plan is in place
- Entering people for repatriation on EBM and liaise with admitting site as appropriate
- Support early discharge identification, monitoring discharges in line with 10am and 12 noon indicators
- Facilitating capacity making actions per Table 3.

Nurse Unit Managers

In addition to the above duties where applicable, the Nurse Unit Managers (NUM) on site will:

- Communicate with Medical Team, Program Manager, site Bed Flow Manager and Nursing Coordinator to complete capacity actions per Table 3. Capacity Making Strategies.





Psychiatric Liaison Nurse (PLN) / Emergency Department Mental Health Liaison Team (ED MHLT) / Consultation Liaison (CL)

The PLN/MHLT is responsible for contacting Bed Flow Managers on site and referring people from ED / general hospital for mental health admission as appropriate. This includes putting the person on EBM and completing required Statewide Standardised Clinical Documentation (SSCD) forms.

Community Mental Health Teams

EMHS community mental health teams will be responsible for:

- Attending inpatient stand up meetings
- Reviewing admissions from community to inpatient as required per capacity making strategies (see Table 3)
 - Consider restricting Community admissions to involuntary admissions (or voluntary where risk is unable to be mitigated in the community) when at Amber status and above.
 - Consultant to consultant decision to admit when at Red BRAG status
- In-reaching into inpatient wards to facilitate early discharges where possible
- Ensuring Treatment, Support and Discharge Plans are in place for community patients needing admission
- Assisting in low-risk transport within EMHS when at Red BRAG status
- Any additional actions per Table 3. Capacity Making Strategies

Consultant

Consultant or senior medical delegate will be responsible for:

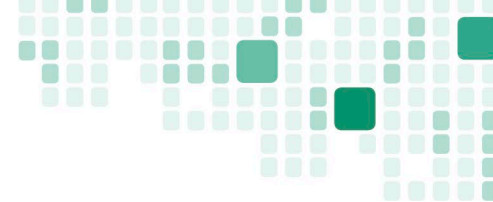
- Reviewing people receiving care in line with site-based procedures
- Liaising with senior nursing staff, and manage bed capacity, or care coordinator capacity (community mental health), for their respective service
- Effectively communicating plans and actions to all members of the multidisciplinary team
- Engaging in consultant to consultant requests for admission from BRAG status Amber
- Ensuring capacity making actions are completed per Table 3.

During business hours, the above is fulfilled by the Consultant allocated to the relevant inpatient unit. After hours, the above is the responsibility of the Consultant on call.

Operations Hub Manager

The Operations Hub Manager is the line manager for the EMHS MH Patient Flow Coordinator. They will have statewide responsibilities per Mental Health Bed Access, Capacity and Escalation Statewide Policy and may delegate this role as required.





Head of Service - Psychiatry / Medical Co-Director

The Head of Service – Psychiatry (AHS) and Medical Co-Director (RPBG) play key roles in escalation within EMHS for mental health bed flow and will fulfil the below roles:

- Point of escalation within mental health for disputes between services within EMHS
- Escalates to the Executive Director Mental Health for matters that cannot be resolved internally
- Involved in capacity making actions and escalation from EMHS Red bed status
- Decision maker in whether to request a general bed (AKG or RPBG) and process for admission
- Decision maker in whether to use an over census bed at their site (where applicable)
- Decision maker in whether to use a leave and/or AWOL bed and process for admission
- Notify Executive Director Mental Health and relevant site Executives
- After hours, the Executive Director Mental Health should be notified via text message of any escalation issues
- Ensures capacity making actions are completed.

When at Black status AKG, RPBG, and SJOGMPH collectively discuss strategy, and alert the Executive Director Mental Health.

Executive Director Mental Health

For AKG and RPBG this refers to the EMHS Executive Director Mental Health, and for SJGMPH this is the Chief Operating Officer. Out of hours the role is fulfilled by the relevant site Executive On Call.

The Executive Director Mental Health will be involved in the mental health bed flow process during escalation processes. In internal disputes, the Executive Director Mental Health will be the point of escalation following unsuccessful negotiation at the Head of Service Psychiatry / Medical Co-Director level.

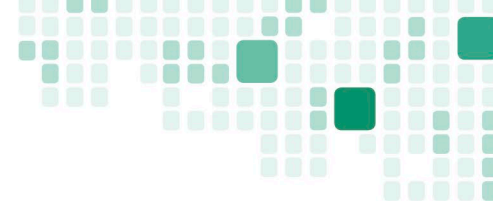
The Executive Director Mental Health will also be informed when EMHS mental health reaches Red or Black bed status and will work with the Head of Service Psychiatry or Medical Co-Director to resolve.

Statewide Nurse Director Mental Health Patient Flow

The Statewide Nurse Director Mental Health Patient Flow has statewide role per the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#). They have daily contact with the HSP Mental Health Patient Flow Coordinators to ensure oversight over all HSP mental health bed capacity and demand.

Bed flow issues that cannot be resolved at the HSP level are to be escalated to the Statewide Nurse Director Mental Health Patient Flow in the first instance.





Statewide Mental Health Medical Director

The Statewide Mental Health Medical Director (MHMD) will act as the escalation point for the Statewide Nurse Director Mental Health, System Flow Centre, and State Health Operations Centre (SHOC) when complex clinical matters related to mental health arise that require systemic intervention and clinical decision making. The MHMD will collaborate with the executive members of the SHOC as well as the executives of HSPs and on-call clinical teams.

The role is held 0800-midnight seven days a week, to ensure that holder is up-to-date with the system pressures and the context of clinical decisions that are present. The role is held as a weekly rotation across HSPs.

Within EMHS, the RPBG Mental Health Division Medical Co-Director and AKG Head of Service Psychiatry, or their delegates, will assume the role of Statewide Mental Health Medical Director when rostered.

Further information about the role of the MHMD can be found in [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#).

Overview of Beds

Statewide bed availability dashboard and information about EMHS bed numbers and referral pathways are linked on the [EMHS Mental Health Bed Access, Capacity and Escalation hub page](#).

Access pathways and process to services

The decision to admit to a specific ward within the service is made as per existing site policy. All requests placed on EBM must be reviewed by the Treating Team prior. A simplified version of the EMHS Mental Health Bed Flow Process can be seen in Figure 1.

Note: The WACHS MH Patient Flow Coordinator will be the single point of contact for transfers in and out of the WACHS regions. The WACHS MH Patient Flow Coordinator will liaise with the EMHS MH Patient Flow Coordinator.

Note: The SJOGMPH Bed Flow Coordinator will be the single point of contact for referrals in and out of SJOGMPH.

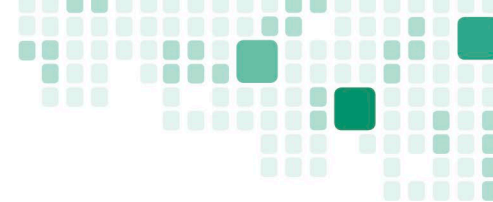
Required Documentation

Referrals to mental health services must include recent and up to date:

- Mental Health Assessment Form (MHAF) or Triage form
- Risk Assessment and Management Plan (RAMP)
- Relevant Mental Health Act forms (if applicable)

All relevant information should also be entered into PSOLIS and the Digital Medical Record / healthcare record.





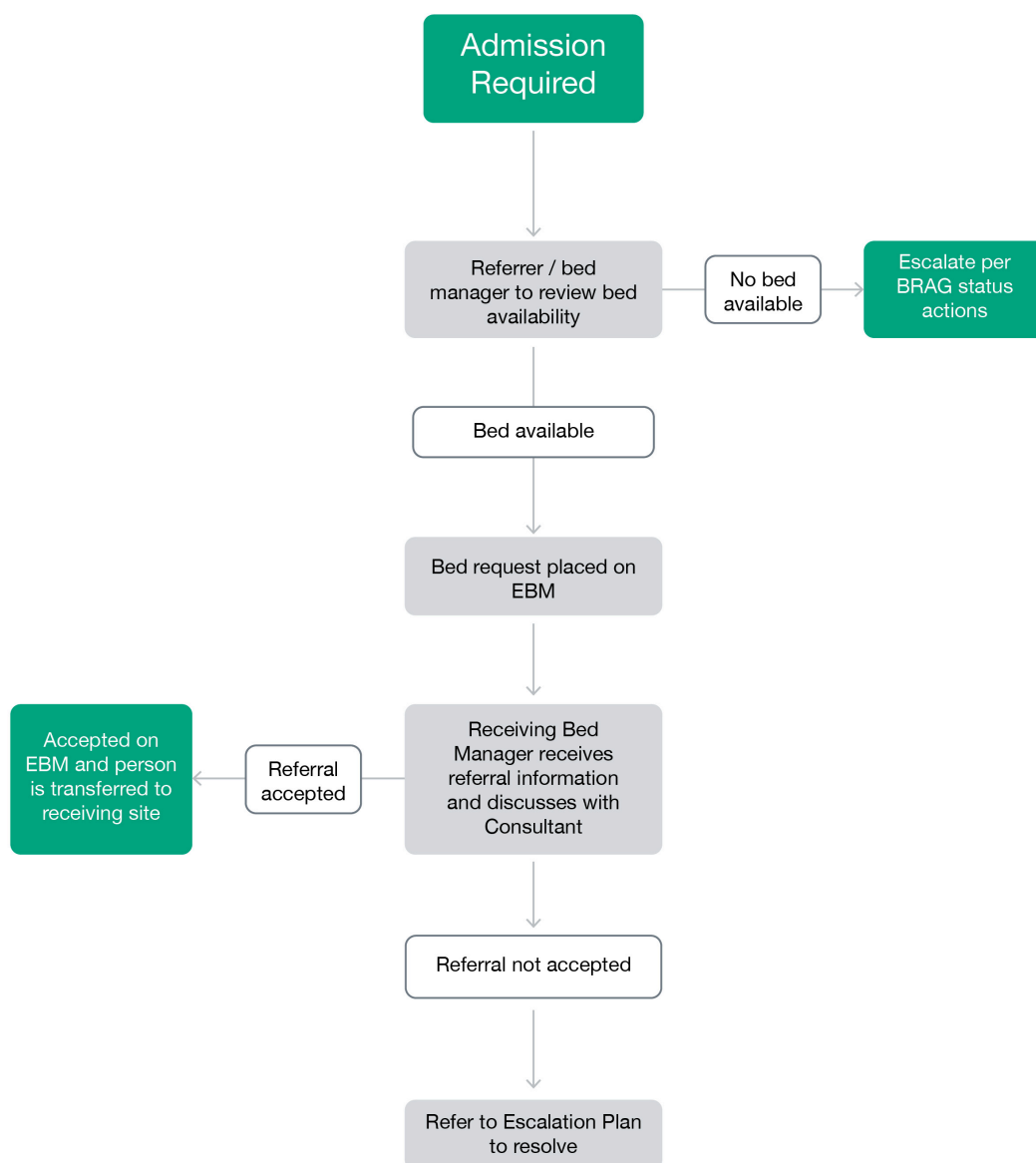
Clinical consideration should be given to referrals received from services who do not use the SSCD (e.g., non HSP-services or services that are exempt from SSCD requirements).

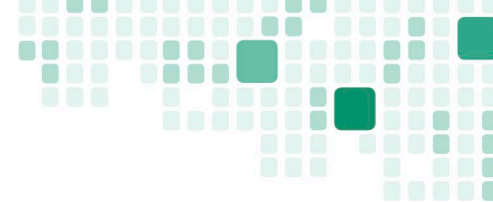
Further documentation requirements can be found in the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#).

Bed Flow Process

Figure 1 provides an overview of the EMHS mental health bed flow process. When referring to/from SJOGMPH and EBM is not available for use, the SJOGMPH Bed Flow Coordinator is the point of contact.

Figure 1: Overview EMHS Mental Health Bed Flow Process





Demand & Capacity

BRAG System

A Black, Red, Amber, Green (BRAG) system is used to indicate bed capacity across each HSP and statewide, per Table 1.

EMHS BRAG will be separated into individual BRAGs for the following cohorts:

- Youth (16 years - 23 years and 365 days)
- Adult (24 years - 64 years and 365 days)
- Older Adult (≥ 65 years; Aboriginal and Torres Strait Islander Older Adults ≥ 55 years)

Table 1. BRAG status

	Green	Amber	Red	Black
	Business as usual	Increasing demand	High demand	Complete bed block
Definition	Business as usual which includes preparatory steps for smooth escalation. Patient flow systems functioning and maintaining performance.	Fewer secure beds than expected. Fewer than expected predicted discharges. Higher number of attendances to EDs and CMH ATTs for planned admissions.	Persistent excess pressure requiring significant management action.	Severe/prolonged excess pressure requiring support from other HSPs and agencies. Significant access block. Unable to place any internal transfers, elective and emergency admissions.
Core business capacity	Adequate capacity to sustain core business	Capacity reduced. Moderate compromise to core business activities.	Capacity reduced. Severe compromise to core business activities	Capacity is compromised. Critical compromise to core business activities.
Demand level	Overall bed availability sufficient to meet predicted demand, including surge.	Overall demand likely to exceed planned discharges, all areas to continue to admit to vacant beds.	Overall demand will exceed planned discharges, but some/most areas continue to admit to vacant beds.	All contingencies fully operational.

The BRAG status is determined as per Table 2 – Demand Status. The demand status may be determined if EMHS is experiencing at least one of the criteria.



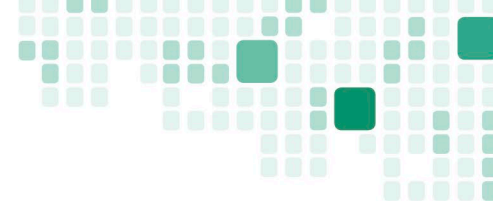


To ensure demand and capacity is accurately recorded, all EMHS mental health inpatient services sites must enter leave (planned, unplanned, and AWOL) over 12 hours in WebPAS against the leave type table in the [WebPAS Reference Guide](#).

Further information about BRAG status can be found in the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#)

Table 2. Demand Status

DEMAND STATUS	
Demand	Criteria
Green (G) Business As Usual	- Six or less people waiting for a non-secure bed across all access points within EMHS (excluding 16-17 year olds) - Three or less people for a secure bed across all access points within EMHS (excluding 16-17 year olds) - Two or less 16-17 year olds waiting for a bed across all access points within EMHS - Any one person waiting in an EMHS ED without an available bed, after being accepted in EBM, for 0-4 hours
Amber (A) Increasing Demand	- Seven to nine people waiting for a non-secure bed across all access points within EMHS (excluding 16-17 year olds) <i>and/or</i> - Three to five people waiting for a secure bed across all access points within EMHS (excluding 16-17 year olds) <i>and/or</i> - Three 16-17 year olds waiting for a bed across all access points within EMHS <i>and/or</i> - Any one person waiting in an EMHS ED without an available bed, after being accepted in EBM, for 4-8 hours
Red (R) High Demand	10 to 11 people waiting for a non-secure bed across all access points within EMHS (excluding 16-17 year olds) <i>and/or</i> - Six to seven people waiting for a secure bed across all access points within EMHS (excluding 16-17 year olds) <i>and/or</i> - Four or more 16-17 year olds waiting for a bed across all access points within EMHS <i>and/or</i> - Any one person waiting in an EMHS ED without an available bed, after being accepted in EBM, for 8-12 hours
Black (B) Complete Bed Block	- Twelve or more people waiting for a non-secure bed across all access points within EMHS (excluding 16-17 year olds) <i>and/or</i> - Eight or more people waiting for a secure bed across all access points within EMHS (excluding 16-17 year olds) <i>and/or</i> - Four or more 16-17 year olds waiting across all access points within EMHS <i>and/or</i> - Any one person waiting in an EMHS ED without an available bed, after being accepted in EBM, for over 12 hours



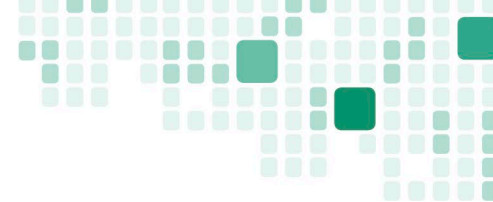
Capacity Making Strategies

Table 3 outlines capacity making actions to be enacted per the BRAG status.

Table 3. Capacity Making Strategies

	CAPACITY MAKING STRATEGIES
GREEN	Capacity Making Actions – Green • Subject to agreed daily protocols • Bed Manager (or equivalent) review of people waiting, beds available and discharges • Bed Managers review to see if people are available for repatriation back to EMHS site if relevant • Discharge list available for prospective planning • Identify any people who are due for discharge and who could be discharged earlier with additional community support • Early identification of discharges and discharges to be in line with 10am and 12 noon indicators • Bed Flow Manager / NUMs / Consultants to agree people suitable for transfer from adult beds to available older adult beds • Bed Flow Manager / NUMs / Consultants to agree people suitable for transfer from EMyU to adult beds vice versa, where appropriate • In addition to St John Ambulance transport, the Wilson patient transport co-ordinator to be notified of inbound and outbound mental health transport needs • Direct admissions from community to inpatient areas, bypassing ED, where possible • Repatriation of people as appropriate
	Communication Channels - Green • Emergency Departments (ED) with patient flow team • ED mental health teams with relevant bed managers • Bed Manager with Ward NUMs and clinicians • Mental Health stand up meetings • Hospital-wide Patient Flow meetings • Bed Manager updates referrer and central team of status through process • Patient flow information recorded in EBM (AKG and RPBG) and webPAS (SJOGMPH) • Metrics around predicted date of discharge (PDD) • Discharge times monitored • Regular site based MH LOS Meetings
AMBER	Capacity Making Actions - Amber • All as per Green status • Expedite timely discharge • Expedite utilisation of off-site beds and/or non-secure beds with specials • Prioritise admissions in accordance with acuity and risk • Utilise transit lounge as available and appropriate • Consider restricting Community admissions to involuntary admissions, or voluntary where risk is unable to be mitigated • CMHS to consider alternative options to ED where appropriate • CMHS to have management plans in place for people needing admission with clear reason for admission and PDD for discharge • Identify people in secure beds suitable for transition to non-secure beds • Utilise leave beds per Bed Usage section below • Consider use of non-specialty bed • Temporary locking of non-secure wards





	Communication Channels - Amber - All as per Green status - Attention to Community, ED, CL - Consultants confirm review of admission requests and priority with Head of Department - Consultant to consultant request for admission
RED	Capacity Making Actions - Red - All as per Amber status - Reporting to confirm unresolved issues have been escalated. - Over-census processes enacted - CMHS admissions only permitted following consultant to consultant discussion - Consider people suitable to be placed on leave - Community teams prepare to assist in low risk transport within HSP - Cancel non-essential meetings - Escalation after hours as per Figure 2
	Communication Channels - Red All as per Amber status
BLACK	Capacity Making Actions - Black - Ensure all previous actions are enacted - Proactively address barriers and escalate to delegated authority as required - Explore all alternative options for non-time critical inter-hospital transfers - HSPs in partnership with patient flow team to identify people suitable for early discharge - Use of alternative bed capacity (non-active, same-day and off-site) - Prioritise acceptance of people under the MHA, and people whose risk cannot be contained in the community setting - Proposed admissions not meeting the criteria above to be negotiated with the Medical Co-Director / Head of Service - CMHS to consider alternative options to inpatient care where appropriate e.g., CRHTT - Liaise with other HSPs to review inbound activity - Notify SHOC System Flow Centre (SFC) and seek assistance as required. Note: All actions to be considered in tandem with each other
	Communication Channels - Black - All as per Amber status - Midland, RPBG and AKG Heads of Service telephone link up to discuss strategy

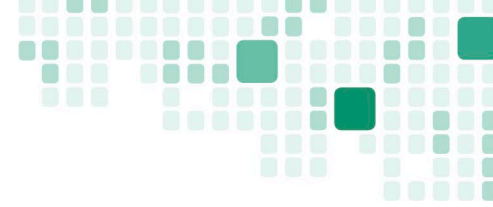
BRAG Black

When a BRAG level Black has been identified there needs to be an assessment on whether the status Black is likely to continue or escalate based on planned discharges and anticipated bed flow.

If all actions in BRAG status Black have been completed and it is deemed by the responsible delegate that the status is likely to worsen, de-escalation protocols should be initiated to resolve the situation, as below:

- Ensure all previous actions and site-specific actions are complete.
- Escalate to the Statewide Nurse Director Mental Health Patient Flow and Mental Health Medical Director as required.
- Inform Executive Director Mental Health to delegate actions to an authorised position for progression in agreed timeframes.
- Notify SHOC SFC and seek assistance as required.





Escalation

Escalation occurs at the HSP level first, then at the system wide level if the HSP has not been able to create sufficient capacity.

System wide Response

System wide responses are only initiated after HSP processes have been exhausted and patient flow issues remain unresolved or when the conditions outlined below are reached.

- Transfer delays (> 4 hours delay)
- HSP to HSP dispute or conflict, where all HSP processes are confirmed as exhausted
- Person waiting for transfer to another HSP is not attended to within 6 hours, and a clear disposition plan is not established within 12 hours
- Admission delay to mental health unit or acute psychiatric unit $\geq$ 10 hours
- System wide BRAG Red or Black

Further information about the systemwide response, including steps to occur by designated role, can be found in Table 1 of the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#).

Catchment Area Considerations

When the system is under pressure or at capacity, all available beds across the system should be taken into consideration for determining the best location for a person requiring care.

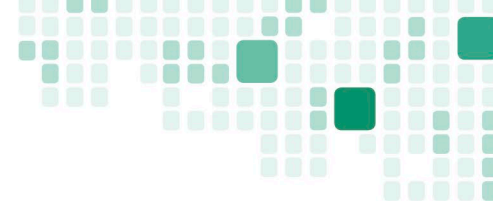
When it is determined that a person is best cared for outside of their catchment HSP, the following must be met:

- The rationale for advocating for admission outside of the person's catchment area is clearly articulated via the bed request, treatment, support and discharge plan (TSDP), and/or crisis management plan
- Safety considerations must be accounted for
- All alternative options have been explored, including review and prioritisation of people admitted and awaiting admission
- Where transfer off Country is required for an Aboriginal person, Aboriginal Health Liaison services and family should be consulted to ensure cultural obligations and wellbeing supports are maintained

The decision to admit outside of the usual HSP is only made following:

- HSP to HSP negotiation (which may or may not include involvement of the MHMD) or
- Direction of the Statewide Nurse Director Mental Health Patient Flow and/or Statewide MHMD:
 - Following disputes between two or more HSPs





- As part of a statewide response.

During periods of significant and unresolvable overcrowding in catchment area EDs or inpatient units, the SHOC System Flow Centre (SFC) will support load levelling by managing the referral and allocation of people requiring care to EDs and inpatient treatment areas outside of their catchment areas (including WACHS).

This includes notifying Referral Site clinical teams and transport services in collaboration with the chosen alternative site. The SFC will coordinate this in partnership with mental health patient flow teams.

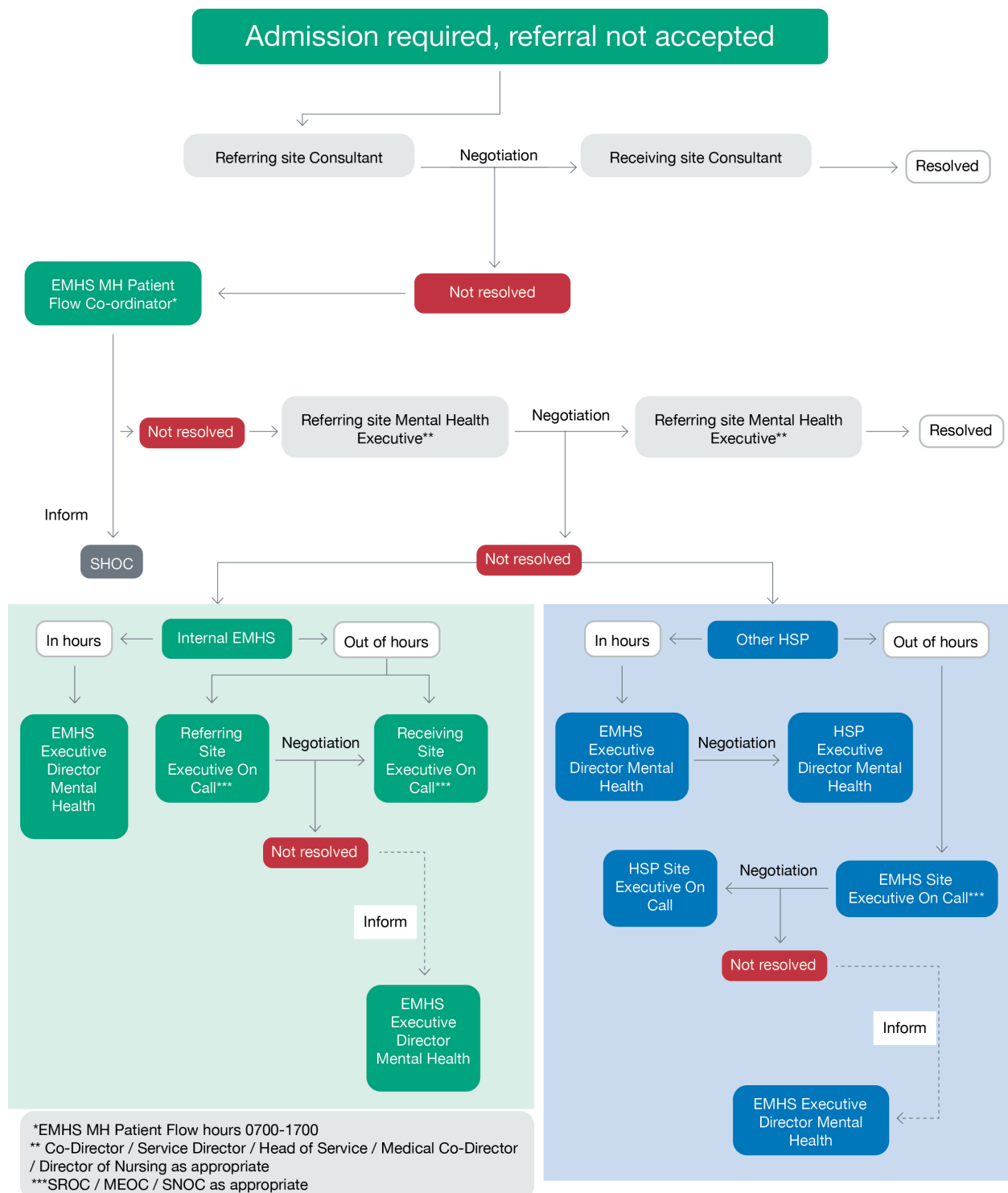
For further information about catchment area considerations, refer to the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#).

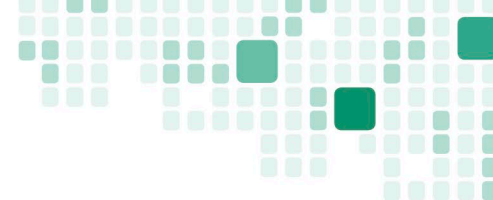


Escalation Pathway

Bed flow escalation within EMHS will be managed per Figure 2.

Figure 2. EMHS mental health escalation pathway





Bed Usage

The below outlines the care pathways for people who are cared for in a bed for mental health reasons against the established business rules. These processes are in line with the Mandatory Business Rules for Bed Management per the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#).

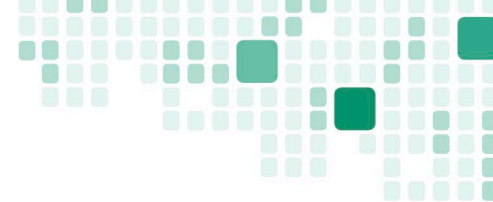
Use of Leave Beds

Steps	Additional Information
1. Applicable circumstances: A <i>leave bed</i> is an available bed when an admitted person is on prolonged leave (overnight or longer).	Consideration of leave bed usage is required to ensure ongoing capacity for people returning from leave. The order of utilisation of leave beds will be in discussion with the treating clinician. To support crossover of care and appropriate follow-up, relevant CMHS to be contacted by the nurse in charge for each person 'on leave' who's bed is utilised. Communication to person going on leave that bed/room may be utilised in their leave period.
2. Exceptions Any leave bed pending transfer from WACHS Any bed pending forensic patient transfer from Frankland Centre	If a person on leave is likely to return before the predicted return time, the Clinical Lead / Service Co-Director or equivalent may quarantine a bed and notify the EMHS MH Patient Flow Coordinator

Use of an Absent With Out Leave (AWOL) bed

Steps
1. Applicable circumstances: An <i>Absent Without Leave Bed (AWOL)</i> is an available bed when the person has been declared AWOL for > 12 hours.
2. Decision to use: Clinical Lead / Head of Department / Medical Co-Director in consultation with ward NUM
3. Communication of use of an AWOL bed: Bed Manager, Program Manager





Over census

Steps

1. Applicable circumstances

An over census bed is used when a person is admitted to an inpatient unit where there is no physical bed capacity. The following criteria should be considered for the ward to accept a person over census:

- the person to be admitted must belong to the specialty of the inpatient unit
- the inpatient unit shift coordinator, in consultation with the NUM, determines which person will be appropriate to be accommodated in the over census beds
- clinical acuity including:
 - nursing skill mix
 - emergency codes in progress
 - specials
 - staffing requirements

All leave beds are to be used prior to considering use of over census beds

2. Decision making to use an over census bed:

During hours – Clinical Lead to discuss with Medical Co-Director / Head of Service who will make decision in consultation with NUM

After hours – On Call Consultant Psychiatrist in consultation with After Hours CNS

3. Communication of use of an over census bed:

Bed Manager, NUM of relevant ward/s

Clinical Prioritisation

Bed prioritisation is to be determined by assessing the greatest clinical need at that point in time. Prioritisation and re-prioritisation of referrals for admission will be considered on the basis of:

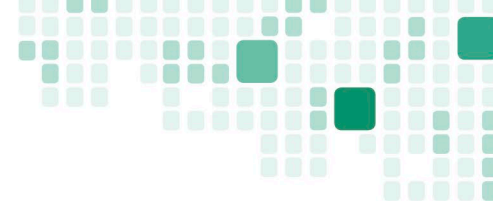
- Acuity
- Degree of containment
- Degree of risk an environment poses to an individual
- Rural and remote

Other instances of high priority can include:

- A person under the Mental Health Act 2014 (MHA 2014) requiring an inpatient bed, including where there is a requirement for a Detention Order (Form 3A)
- When transfer to a mental health bed has been delayed for more than 24 hours

An admission may also be re-prioritised if the person has waited an unacceptable length of time (noting a statewide response may be triggered when a person has been waiting



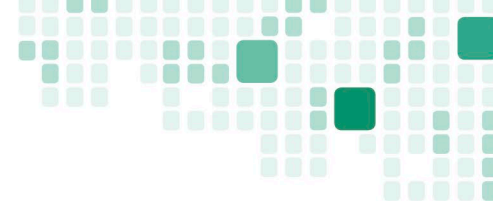


for admission and transfer to a Mental Health Unit for 10 hours or more). A referral that has been waiting an unacceptable length of time will be raised with the EMHS Patient Flow Coordinator.

The decision to escalate the priority of an admission, regardless of time waiting, will be made by a Consultant Psychiatrist and communicated to the EMHS Patient Flow Coordinator.

Refer to Appendix 1 of the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#) for further details on prioritisation.





Appendix 1: WACHS catchment areas

WACHS Region	Tertiary Health Service Code	Tertiary Hospital
Kimberley	EMHS	RPH
Pilbara		
Wheatbelt *		
Goldfields	NMHS	SCGH
MidWest		
Wheatbelt *		
Great Southern	SMHS	FSH
South West		

Health District	Area	Tertiary Health Service Code	Tertiary Hospital
Coastal Wheatbelt	Chittering	EMHS	RPH
Coastal Wheatbelt	Toodyay		
Eastern Wheatbelt	Cunderdin		
Eastern Wheatbelt	Merredin		
Eastern Wheatbelt	Mukinbudin		
Southern Wheatbelt	Brookton		
Southern Wheatbelt	Murray		
Western Wheatbelt	Dowerin		
Western Wheatbelt	Moora		
Western Wheatbelt	Northam		
Western Wheatbelt	Toodyay		
Western Wheatbelt	York - Beverley		
Coastal Wheatbelt	Gingin - Dandaragan	NMHS	SCGH
Southern Wheatbelt	Brookton	SMHS	FSH
Southern Wheatbelt	Kulin		
Southern Wheatbelt	Narrogin		
Southern Wheatbelt	Wagin		



